

Metadata Schema Coordination Meeting

1. Meeting Goal

The goal of this meeting is to explore how different initiatives working on **metadata schemas** can align efforts and potentially coordinate development for **Post-acute infection syndromes (PAIS)** in the context of the **European Health Data Space (EHDS)**.

Specifically we aim to:

- Share the **current status of metadata schema development** in each initiative
- Identify **overlaps and differences**
- Explore whether a **shared or aligned metadata framework** could be developed
- Discuss potential **next steps for coordination**.

Please add comments or input directly in this document before the meeting

2. Meeting Information

Doodle for scheduling:

<https://doodle.com/group-poll/participate/aQKKPpId>

Proposed meeting window:

10:00 - 11:00 9th April 2026

Next meeting with Shuxin & Hajar: April 29, 2026

Organizer:

Shuxin Zhang

Supporting organizer:

Margreet

3. Participants and Perspectives

Perspective	Initiative / Organization	Representative
EHDS perspective (DCAT-AP)	Health-RI	Hannah
Post-COVID perspective	PCNN	Bianca, Hajar
ME/CFS perspective	NMCB	Jos, Shuxin

Biobank perspective	BBMRI.nl	Peggy
Funder perspective	ZonMw	Margreet

4. Current Work (Please Add Brief Updates)

Each initiative is invited to add a short update (3–5 bullets).

EHDS/Health-RI

- Current activities:
 - HRI metadata schema aligned with EHDS (HealthDCAT-AP)
 - Implement HRI metadata schema in catalogue
 - Onboard diverse group of data holders (umcs + domain nodes + individual organisations)
- Relevant frameworks: HealthDCAT-AP
- Key challenges:
 - Finding consensus in how to fill in metadata schema
 - Supporting data holders in onboarding
 - HealthDCAT-AP is quite general → does not fulfill needs of all dataholders
- [Health-RI Metadata Model](#)
 - Harvesting metadata from FDPs at umc's or the central FDP at HRI
 - Or push from Dataverse
- Onboard email: onboarding@health-ri.nl

PCNN

- Current metadata work:
 - [Metadata collected of 32 \(pre-PCNN\) studies \(shuxin: variable level\)](#)
 - [Human readable metadata format available through PCNN-Projects/Metadata-format](#) based on EMIF metadata format (shuxin: study level)
 - Health RI checked **overlap** our metadata format and needed metadata for EHDS - there is overlap, but more differences
 - Exploring **examples** of online, searchable metadata catalogi: YODA, CKAN, Molgenis
- Existing standards used: for now only in codebook:
 - [Snomed ct](#)
 - [Loinc](#)
 - [NCIT](#)
 - [\(hajar?\)](#)
- Key needs:
 - [Discuss how to align metadata, codebook, studymanagement](#)

- How to take metadata into account in the codebook? Metadata is on a different level than individual variables, namely on study level (aligns more with desired studymanagementsystem?). Should these be in the same codebook and are ontologies needed for all study level variables?
- Discuss the optimal way: where to collect which metadata? UMCs will be obliged under EHDS to report metadata of their studies, where to draw the line? Aim: collect enough for PCNN, but avoid double work for researchers
- Will there be a PAIS-node? If so, where exactly?

NMCB

- Current metadata work:
 - NMCB codebook available
 - No existing metadata schema
 - Demand for 'organizing' our metadata

BBMRI.nl

Metadata for biobanks

- Metadata model: which attributes are used to describe the data and how should they be provided?
 - Mandatory and recommended information
 - Format: human- and machine-readable
 - Controlled vocabularies
- Health-RI metadata model: general metadata model
 - Aligned with EHDS metadata model (HealthDCAT-AP)
 - General + health-related fields
 - Not specific for biobanks
- Current agreement: 1 catalogue entry per Biobank or Collection in the National Health Data Catalogue
 - Via onboarding route of the umc
 - For multi-center projects: via coordinating center
 - Agreements about how to provide specific information:
 - Available biosamples
 - Population specifics

Note:

- information regarding FAIR Data Point Amsterdam: <https://fdp.healthdataspace.amsterdam>
- Contact Erik van Iperen (e.p.vaniperen@amsterdamumc.nl) en Rene Oostergo (r.j.oostergo@umcg.nl) for more detailed information.
- Existing biobank metadata standards

ZonMw

- Expectations from a funder perspective:
 - Current: Search for options
 - to reuse existing data,

- To use domain-specific standards,
 - To engage a data steward,
 - To ensure sufficient budget for DS & ds-activities.
- DMP
- Under development: machine-actionable DMP and PIDs
- A minimal set of md to enrich the description of datasets (number of participants/subjects, variables,
- Interoperability priorities:
 - Controlled vocabulary for
 - generic health terms (EHDS compliant) and
 - (to be developed) detailed terms for rich description of health/disease,
 - topics to describe context.
 - PIDs (ROR, ORCID, doi)

5. Existing Resources (Please Add Links)

Participants are encouraged to add relevant documents, schemas, or frameworks.

Example:

Resource	Link	Comment
Health-RI metadata schema	https://github.com/Health-RI/health-ri-metadata	
Health-RI Data Catalogue	https://catalogus.healthdata.nl/en	
PCNN metadata dictionary	https://github.com/PCNN-Projects/Metadata-format	
NMCB variable list	NMCB Codebook	
BBMRI MIABIS standard	https://www.bbmri-eric.eu/how-tomiabis/	
Metadata schemes for covid-19 (2021-2025):	M4M - GO FAIR Foundation	
- Project admin - Project content - Collection (e.g. biosamples) - Dataset (dcat)		
Bioportal	ZonMW COVID-19 NCBO BioPortal	

[ZonMw Generic Terms | NCBO BioPortal](#)

Nanopublication templates [Publish a new Nanopublication | nanodash](#)
[Publish a new Nanopublication | nanodash](#)

Renske: see the initiatives that are mentioned in grant proposals Not to be linked yet Create an overview

Bianca: in PCNN currently a inventory is being performed to see who is participating in a HORIZON call

DCAT metadata schema <https://www.w3.org/TR/vocab-dcat-3/#basic-example>

6. Key Questions for the Meeting

Add questions to discuss:.

- What **minimum metadata set** should exist across PAIS cohorts?
- How can PAIS cohorts align with **EHDS requirements**?
- Can we reuse existing standards such as **MIABIS or other health metadata frameworks**?
- Should we aim for a **shared schema or mapping between schemas**?
 - Margreet: A minimal set of common md-elements
 -
- How can metadata schema be translated into tangible products such as data catalogue?

Margreet:

- Develop an input-form for researchers to provide md
- Catalogue to expose the md (complementing the EHDS national catalogue)
- Search-find-request-procedure to request data access (hdab?)
- Data access - terms of use for data
- Predefined Sparql-queries to analyse collected (meta)data for knowledge generation
- visualizations

Add additional questions below.

7. Expected Outcomes of the Meeting

Potential outcomes may include:

- Agreement on **shared metadata principles**
- Identification of **common metadata domains**
- Planning md-actions
 - Margreet suggests:
 - Meet to decide on the **metadata elements** within a md-scheme for a more detailed description of the project content
 - Use the **terms/controlled vocabularies** that are already developed in the PCNN-codebook end biobank-metadata
 - Reske suggests:
 - Find similar initiatives in national and EU to collaborate
- Who will participate, roles

8. Agenda (Draft – to be updated)

1. Introduction and meeting objective (5 min)
2. Short updates from each initiative
 - a. PCNN + NMCB (PAIS)
 - b. BBMRI
 - c. Health-RI
3. Discussion on overlaps and alignment opportunities
4. Discussion on EHDS relevance
5. Agreement on next steps
 - a. In May 2026 > Shuxin will send out a doodle

Bianca: make it for researchers as easy as possible, FAIR Data Point is too complex, how PCNN network stands for Health-RI Data Catalogue: <https://catalogus.healthdata.nl/en>

We can create a **PAIS node in Health-RI**, but there is no funding yet.

Next step: April 29, 2026

Shuxin & Hajar & Margreet

We must set a feasible scope for the mecvs/PCNN-metadata-development, considering:

- Health-dcat-ap-NL
- Responsibilities of partners: role - support - development:
 - Health-RI >> ask their support for issues that are part of THEIR responsibility!
 - Funder / ZonMw >> identifies needs and initiatives solutions, but other must carry out
 - Consortium & funded projects (ZonMw, mecvs, pcnn, pais)
 - Other stakeholders who have interest in well established metadata system
- Available experts, time, budget

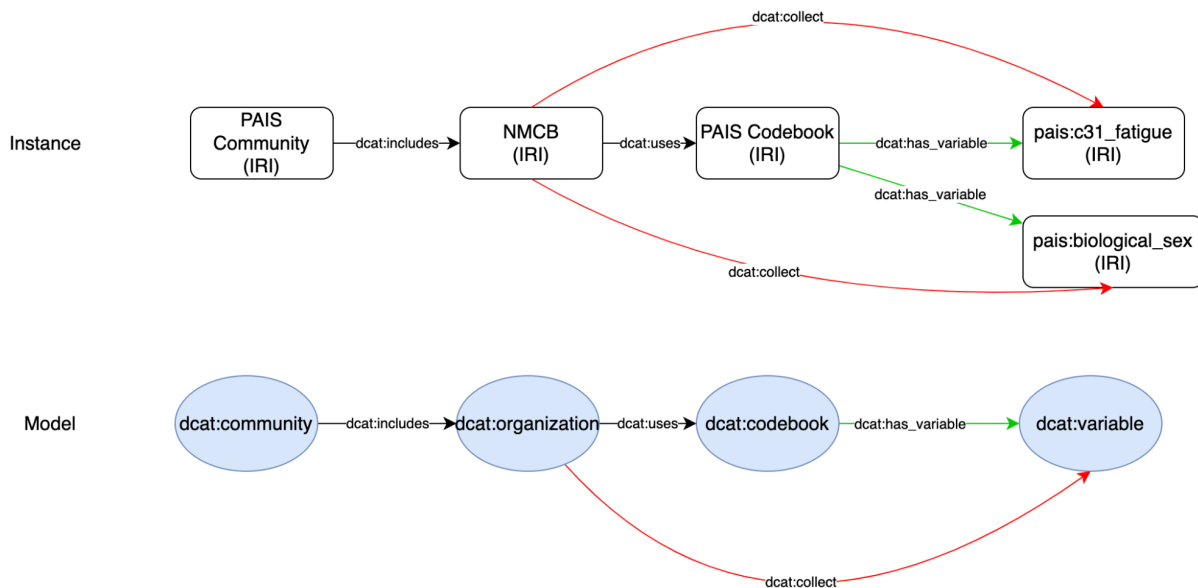
Metadata system:

- Metadata scheme
 - Health-dcat-ap-nl > use this as much as possible. For additional metadata, create your own schema.
 - Additional md-schemes, 'other md-scheme' (create/follow a standard > which one?)
 - Controlled vocabularies
 - Does health-dcat provide all the attributes (in recommended set) for a rich description of data?
 - I miss an attribution in health-dcat for 'variables'
 - What we miss, can be connected >>
 - Connecting other md-schemes to Health-dcat-ap-nl with help of:
 - Dcat: qualified attribution (roles, e.g. funder,)
 - Dcat: qualified relation (e.g. connect the codebook
 - PIDs: grant-id, orcid, ror, doi
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- Metadata input form
 - Onboarding HRI
 - maDMP ZonMw (under development)
 - Additional md-schemes for 'other md-scheme' with m.a.endpoint / e.g. FDP, nanopublication (to be developed by institute, consortium, or
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- Metadata exposure > md-catalogue, data platform, website
 - National Catalogue

To do's:

- Margreet wants to ask Mijke/Hannah how we can work according to the above.

- Shuxin/Hajar will be available to explain the use case with the code book, xls of PCNN, onboarding process
- The pcnn metadata dictionary xls: It can be seen as an entry form for researchers, but we must discuss with HRI how to align it with H-dcat-md-scheme > to be discussed with HRI/Mijke.
- Codebook PAIS > extra info to be added by Shuxin & Hajar:
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- **Shuxin focusses:**
 - on the documenting his work before he leaves for his next position;
 - Onboarding NMCB
- **Shuxin rough thoughts about connection between Health-DCAPT-AP-NL and codebook:**
 - The simple way:
 - <NMCB> <dcat:uses> <link of PAIS Codebook>
 - The advanced way:
 - <NMCB> <dcat:collect> <c31_fatigue>
 - <NMCB> <dcat:collect> <biological_sex>
 -



One mapping: **pcnn:fatigue** → mapped to → **sct:fatigue (sct:123456)**

FAIR variable name:

- sct_12345 → <<http://snomed.info/id/>12345>
- ncit_2356667

URI - **sct:12345**

PREFIX sct: <<http://snomed.info/id/>>

PREFIX ncit: <http://purl.obolibrary.org/obo/NCIT_>

Sct:fatigue → <http://snomed.info/id/fatigue>

Different mapping relations:

1. **pcnn_fatigue** → **sct_fatigue**
 - a. **pcnn:fatigue** → **skos:exactMatching** → **sct:fatigue**
2. **Pcnn_sleeping_tiredness_10_days** → **sct_sleeping_tiredness_{10d}**
 - a. **pcnn:sleeping_tiredness_10_days** → **skos:narrowerthan** → **sct:sleeping_tiredness** (WE DO NOT CREATE ONE FOR SNOMED CT)
 - b. **Pcnn:sleeping_tiredness_10_days** → **skos:exactMatching** → **sct:sleeping_tiredness_10_days** (When we ask SCT to create a new one)
3. **pcnn:fatigue** → **skos:exactMatching** → **sct:fatigue**

pais:mapping01 rdf:type dcat:mappings

dcat:has_subject **pcnn:fatigue**

dcat:has_predicate **skos:narrowMatching**

dcat:has_object **sct:fatigue_related**

dcat:author ocird:hajar

dcat:status "wait for SNOEMD CT new codes"